



Placement and Residential Licensing Unit

P.O. Box 1437, Slot S140, Little Rock, AR 72203-1437

P: 501.682.8590 F: 501.683.6060 TDD: 501.682.1550

Notice of Serious Incident

Case Number: 023824

Date of Incident: 9/5/2024

Date Received: 9/6/2024

Facility Name: Youth Home, Inc.

Facility Number: 128

Incident Type: Licensing

Report Description: Incident Report for [REDACTED] client in our PRTF program and resides in Sturgis House Incident report date/time: 09/05/24 11:30am Location of Incident: Siebert School Incident Description: Medical Emergency (Trip to ER/Urgent Care) Staff Involved: Valerie Alvarez, Angela Shy Events Leading: [REDACTED] was seen today for routine weekly medication management and evaluation. He discussed the physical altercation that occurred over the weekend. After asking direct questions, it was determined he will be sent to ACH ER to rule out concussion or other injuries. Nursing Assessment date/time: 09/05/24 11:30am: [REDACTED] was seen today for routine medication management and evaluation. He reported to be "good" today. He reports his week is getting "better". Denies peer conflict. He reports he can get to sleep but wakes throughout the night. Feels the Vyvanse 10mg "definitely helps". Discussed he feels he could benefit from bumping the dose up. Discussed past medications trials. Discussed fight that occurred over the weekend. He feels that he and the peers that hit him have restored and he feels safe around them. He discussed he is mostly upset with the 2 that instigated the altercation. He has a mild headache, predominantly on the right side of his head, that has remained. He admits only mentioning this to the nurse the day of the incident and received apap. Upon further questioning, he endorsed blurred vision out of (R) eye and a popping/clicking jaw. Denies any nausea, vomiting, weakness, numbness, tingling, fatigue, sensitivity to light or noise, or tinnitus. He reported pain on inhalation on the right lower back, no obvious bruising noted, and no difficulty taking a breath. No signs of respiratory distress, retractions, accessory muscle use, or nasal flaring. No visible signs of trauma, erythema, bruising, abrasions, lacerations, deformities, deviations, protrusions, or edema. No signs of trauma or abnormalities to oral mucosa. To be noted: there is some sort of hyperpigmentation/scar tissue noted above his (R) eye in which he reports is the result of an older injury. Denies difficulty eating/swallowing/talking. No palpable hematomas were noted to scalp. He answers appropriately. Clear, calm coherent speech. AAOx4. PERRLA.

Grip strength bil equal, +2. Capillary refill brisk, less than 3 seconds, and WNL. Gait is steady. Discussed when at home, he usually takes 800mg of IBP once daily. No further concerns or physical complaints voiced. Due to c/o continued headache, clicking/popping jaw, and blurred vision out of (R) eye, MD ordered for [REDACTED] to be transported to ACH to rule out concussion or any other sustained injuries. increase in addition to increasing trazodone 150mg QHS and schedule ibuprofen 800mg BID x3 days Guardian was notified on 09/05/24 at 1:15pm ER paperwork was emailed to Kendra Slade on 09/06/24

Interim Action Narrative: Resident was assessed and transported to ACH for further evaluation.

Maltreatment Narrative:

Licensing Narrative: Licensing reviewed provider reported incident for licensing concerns. Facility provided documentation for this incident.